

Having Tough Conversations with Families of Children Who Require Additional Support

Participant Handout & Template Packet

Professional Development for Early Childhood Educators and Child Care Providers

Childcare Excellence Association Training Session



**CHILDCARE
EXCELLENCE**

ASSOCIATION

PROFESSIONAL DEVELOPMENT FOR EARLY CHILDHOOD PROFESSIONALS

Sample Observation Templates & Worksheets

Use these templates to document observations before speaking with families.

TEMPLATE 1: General Child Observation Form

⚠ **Document facts, not assumptions. Describe what you see and hear. Do not diagnose.**

Child's Name or Initials:

Date:

Time:

Location or Activity:

Observer (Staff Initials):

Observation Record

What happened immediately before (Antecedent):

What the child did or said (Objective Observation):

How the adult responded (Consequence / Adult Response):

What helped or did not help:

Duration or frequency of behavior/response:

Follow-up notes or patterns noticed:

CONFIDENTIALITY: This record contains personally identifiable information. Store securely, share only with authorized personnel, and follow your program's confidentiality policies and NYS OCFS regulations.

ABC Behavior Observation Form

ABC = Antecedent | Behavior | Consequence

⚠ Document facts, not assumptions. Describe what you see and hear. Do not diagnose.

Child's Name or Initials:

Date:

Time:

Location or Activity:

Observer (Staff Initials):

A — ANTECEDENT (What happened before?)	B — BEHAVIOR (What did the child do or say?)	C — CONSEQUENCE (What happened after? How did adults respond?)

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What helped or did not help:

Follow-up notes:

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TEMPLATE 3: Frequency Count Form

Use this form to track how often a specific behavior or event occurs.

⚠ Document facts, not assumptions. Describe what you see and hear. Do not diagnose.

Child's Name or Initials:

Date:

Location or Activity:

Observer (Staff Initials):

Specific behavior being tracked (describe objectively):

Tally each occurrence in the boxes below:

Time of Day	Activity / Routine	Tally Count (mark each occurrence)	Notes

Total occurrences today:

Pattern noticed over time:

Strategies tried and child's response:

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TEMPLATE 4: Transition Observation Form

Use this form to observe a child's responses during transitions between activities or routines.

⚠ Document facts, not assumptions. Describe what you see and hear. Do not diagnose.

Child's Name or Initials:

Date:

Observer (Staff Initials):

Transition	Child's Response (objective)	Duration / Intensity	What Helped
Arrival			
Morning circle			
Snack			
Outdoor / indoor transition			
Lunch			
Rest time			

Activity change			
Dismissal / departure			

Overall pattern noticed:

Follow-up notes:

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TEMPLATE 5: Social-Emotional Observation Form

Use this form to document observations related to peer relationships, self-regulation, emotional expression, and sense of self.

⚠ Document facts, not assumptions. Describe what you see and hear. Do not diagnose.

Child's Name or Initials:

Date:

Time:

Location or Activity:

Observer (Staff Initials):

Peer interactions observed:

Emotional responses observed:

Self-regulation strategies observed:

What triggered strong emotional responses (describe factually):

What helped the child feel calm or regulated:

Relationships with specific peers or adults observed:

What helped or did not help:

Follow-up notes:

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TEMPLATE 6: Communication Observation Form

Use this form to document observations related to the child's use of language, gestures, and communication attempts.

⚠ Document facts, not assumptions. Describe what you see and hear. Do not diagnose.

Child's Name or Initials:

Date:

Time:

Location or Activity:

Observer (Staff Initials):

Child's primary communication methods (words, gestures, pointing, sounds):

Approximate number of words or phrases used during observation period:

Examples of communication attempts (what the child did or said):

How others responded to the child's communication:

Communication that was successful (child was understood):

Communication that was challenging (child was not understood):

What helped or did not help:

Follow-up notes:

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TEMPLATE 7: Sensory Processing Observation Form

Use this form to note how the child responds to sensory input in the environment.

⚠ Document facts, not assumptions. Describe what you see and hear. Do not diagnose.

Child's Name or Initials:

Date:

Time:

Location or Activity:

Observer (Staff Initials):

Sensory inputs that appeared comfortable for the child (describe):

Sensory inputs that appeared uncomfortable or distressing (describe):

Child's response to touch, textures, or materials:

Child's response to sounds or loud environments:

Child's response to visual stimulation or lighting:

Movement or body awareness observations:

What helped or did not help:

Follow-up notes:

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TEMPLATE 8: Physical or Mobility Observation Form

Use this form to document observations related to gross motor, fine motor, or physical functioning.

⚠ Document facts, not assumptions. Describe what you see and hear. Do not diagnose.

Child's Name or Initials:

Date:

Time:

Location or Activity:

Observer (Staff Initials):

Gross motor skills observed (climbing, running, jumping, balance):

Fine motor skills observed (grasping, drawing, cutting, buttoning):

Physical assistance needed during daily routines:

Child's response to physical activity:

Accommodations or modifications currently in place:

What supported greater independence or participation:

What helped or did not help:

Follow-up notes:

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TEMPLATE 9: Attention and Learning Observation Form

Use this form to document observations related to attention, focus, task completion, and learning approaches.

⚠ Document facts, not assumptions. Describe what you see and hear. Do not diagnose.

Child's Name or Initials:

Date:

Time:

Location or Activity:

Observer (Staff Initials):

Length of time child engaged in child-directed play:

Length of time child engaged in adult-directed activities:

Strategies used to maintain or regain focus:

Types of activities or materials that held child's interest longest:

Observations during structured group learning:

What helped child complete tasks or activities:

What helped or did not help:

Follow-up notes:

CONFIDENTIALITY: This record contains personally identifiable information. Store securely, share only with authorized personnel, and follow your program's confidentiality policies and NYS OCFS regulations.

Documentation Examples: Subjective vs. Objective Language

Use these examples to guide professional documentation practices.

Objective documentation describes what you observe — what the child does and says, when, where, how often, and for how long. It does not include labels, diagnoses, interpretations, or judgments.

Example 1: Social-Emotional Development

✗ Instead of writing:	✓ Write instead:
<i>Mia is very emotional and dramatic. She can't handle it when she doesn't get her way.</i>	During free play on three separate mornings this week, Mia cried for 8–12 minutes when another child took a toy she was using. She said 'That's mine!' and lay on the floor. She did not respond to verbal redirection during the first 5 minutes of each episode.
Why this is better: The objective version describes specific, observable actions including time, frequency, and exact words. It does not label the child or interpret the behavior. Anyone reading this record can understand exactly what occurred.	

Example 2: Physical or Mobility

✗ Instead of writing:	✓ Write instead:
<i>He has motor problems and can't do anything the other kids do.</i>	During outdoor time, Devon was unable to climb the ladder to the slide independently. He reached the third rung and stopped on 4 out of 5 attempts. He completed the slide with adult hand support. He did not participate in the running game.
Why this is better: The objective version describes specific, observable actions including time, frequency, and exact words. It does not label the child or interpret the behavior. Anyone reading this record can understand exactly what occurred.	

Example 3: Attention and Learning

✗ Instead of writing:	✓ Write instead:
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<i>She is so hyper and can never focus.</i>	During the 20-minute morning circle, Priya left her spot 6 times. She returned when a teacher stood beside her. During small group activities with manipulatives, she remained seated and engaged for 12 minutes continuously.
Why this is better: The objective version describes specific, observable actions including time, frequency, and exact words. It does not label the child or interpret the behavior. Anyone reading this record can understand exactly what occurred.	

Example 4: Sensory Processing

✗ Instead of writing:	✓ Write instead:
<i>He freaks out during lunch because of his sensory issues.</i>	On 8 of the past 10 school days during lunch, Carlos pushed away foods with soft or mushy textures after one or two bites. He said 'yucky' or 'no' and covered his mouth. He ate crackers, carrots, and apple slices without difficulty.
Why this is better: The objective version describes specific, observable actions including time, frequency, and exact words. It does not label the child or interpret the behavior. Anyone reading this record can understand exactly what occurred.	

Example 5: Communication

✗ Instead of writing:	✓ Write instead:
<i>She barely talks. She's really delayed.</i>	During a 30-minute observation of free play, Nadia communicated primarily using gestures (pointing, reaching) and 3–4 single words. She did not yet use two-word combinations during this observation. She responded to her name 3 out of 5 times.
Why this is better: The objective version describes specific, observable actions including time, frequency, and exact words. It does not label the child or interpret the behavior. Anyone reading this record can understand exactly what occurred.	

Example 6: Challenging Behavior During Transitions

✗ Instead of writing:	✓ Write instead:
<i>He's defiant every time we clean up. He's manipulative.</i>	During clean-up time on Monday, Tuesday, and Wednesday, Eli sat under the table and said 'I don't want to' when given the clean-up

	direction. He remained under the table for 5–7 minutes on each day. He joined the group after a teacher sat beside him and offered a specific task.
Why this is better: The objective version describes specific, observable actions including time, frequency, and exact words. It does not label the child or interpret the behavior. Anyone reading this record can understand exactly what occurred.	

Example 7: Peer Interaction

✗ Instead of writing:	✓ Write instead:
<i>She has no friends and doesn't know how to play with other kids.</i>	Over the past three weeks, Sofia played independently or beside peers on 14 out of 15 observed days. On 1 day, she entered a peer's play and stayed for 4 minutes before leaving. She has not yet initiated conversation with peers during observation periods.
Why this is better: The objective version describes specific, observable actions including time, frequency, and exact words. It does not label the child or interpret the behavior. Anyone reading this record can understand exactly what occurred.	

Conversation Planning Worksheet

Complete this worksheet before speaking with a parent or caregiver.

Using this worksheet helps you feel prepared, organized, and confident before the conversation.

Child's Name or Initials:

Child's Age:

Classroom or Group:

Date of Planned Conversation:

Meeting Location:

Staff Member Completing This Worksheet:

Section 1: Child's Strengths

List at least 3 genuine strengths to share at the start of the conversation:

Strength 1:

Strength 2:

Strength 3:

Section 2: Reason for This Conversation

What concern or observation prompted this meeting?:

What is your primary goal for this conversation?:

Section 3: Objective Observations to Share

List 2–4 specific, documented observations (include dates, times, and details):

Observation 1:

Observation 2:

Observation 3:

Observation 4:

Section 4: Patterns Noticed Over Time

How long have you been noticing this? Across what settings or routines?:

How frequently does this occur?:

Section 5: Questions for the Family

Sample prompts to use:

- "I'd like to share some observations and hear what you are noticing at home."
- "My goal is to work together to support your child."
- "Have you seen anything similar in other settings?"
- "What strategies work well for your child at home?"
- "What does your child enjoy most? What are their strengths at home?"
- "Is there anything going on at home that I should know about?"

Your own questions to ask this family:

Question 1:

Question 2:

Question 3:

Section 6: Supports Already Tried

What have you already tried in the program?:

What has helped?:

What has not helped?:

Section 7: Suggested Next Steps

What will you do in the program?:

What might the family try at home?:

Possible referrals or resources to mention:

Follow-up plan (when will you reconnect?):

Section 8: Notes From the Conversation

Complete this section after the conversation:

Family's response and perspective:

What was agreed upon?:

Next steps confirmed with family:

Date of follow-up:

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Local and State Support Services List — New York State

Customize this list with contacts specific to your county, school district, and community.

Providers should customize this list with local county, school district, and program-specific contacts. Sharing this list with families should be done in partnership with your program director and in accordance with your program's policies.

EARLY INTERVENTION (EI) PROGRAM

Purpose: Provides services for children with developmental delays or disabilities.

Who It Serves: Children from birth through 35 months of age.

When to Consider: When you observe persistent delays in communication, motor, cognitive, social-emotional, or adaptive development.

What You May Need: Child's name, date of birth, address, and description of concerns. Parent's contact information. Providers may refer without parent consent but must notify the family.

Your local contact:

COMMITTEE ON PRESCHOOL SPECIAL EDUCATION (CPSE)

Purpose: Evaluates and provides special education services for eligible preschoolers.

Who It Serves: Children ages 3 through 5 who reside in the school district.

When to Consider: When a child turns 3 or when significant concerns are identified during preschool years.

What You May Need: Parent's written consent is required for evaluation. Contact your local school district's CPSE chairperson.

Your local contact:

PEDIATRICIAN OR PRIMARY HEALTH CARE PROVIDER

Purpose: Conducts developmental screening and can refer to specialists.

Who It Serves: Children of all ages.

When to Consider: Always a first step. Share your observations with the family and suggest discussing them at the child's next well-child visit.

What You May Need: Family's permission to share observations. Written documentation of what you have observed.

Your local contact:

LOCAL SCHOOL DISTRICT

Purpose: Responsible for CPSE evaluations and services for eligible preschoolers.

Who It Serves: Children ages 3–21 with disabilities.

When to Consider: When a child approaching age 3 may need an evaluation, or when a school-age concern arises.

What You May Need: Family contact with their home school district. CPSE Chairperson name and phone number.

Your local contact:

CHILD CARE RESOURCE AND REFERRAL AGENCIES (CCR&R)

Purpose: Supports child care providers and connects families to local resources.

Who It Serves: Providers and families across New York State.

When to Consider: When you need training, coaching, or help connecting a family to community resources.

What You May Need: Contact your regional CCR&R agency — find yours at www.ocfs.ny.gov.

Your local contact:

NYS OFFICE OF CHILDREN AND FAMILY SERVICES (OCFS)

Purpose: Regulates child care programs and provides policy and training resources.

Who It Serves: All licensed and registered child care providers in NYS.

When to Consider: When you have questions about your regulatory requirements, documentation standards, or licensing guidance.

What You May Need: Contact your regional OCFS office or visit www.ocfs.ny.gov.

Your local contact:

NYSED PRESCHOOL SPECIAL EDUCATION RESOURCES

Purpose: Provides guidance on preschool special education law, policy, and practice.

Who It Serves: School districts, families, and service providers.

When to Consider: When seeking policy clarification about CPSE or preschool special education rights.

What You May Need: Visit www.nysed.gov/special-education.

Your local contact:

DEVELOPMENTAL SCREENING RESOURCES

Purpose: Standardized tools such as the ASQ-3 and M-CHAT help identify developmental concerns.

Who It Serves: Children of all ages depending on the tool.

When to Consider: As a supplement to your observations — share screening information with the family and suggest follow-up with a pediatrician.

What You May Need: Permission to administer screenings varies by program. Check with your director.

Your local contact:

MENTAL HEALTH CONSULTATION RESOURCES (ECMHC)

Purpose: Early Childhood Mental Health Consultation supports children, families, and providers in addressing social-emotional and behavioral concerns.

Who It Serves: Children, families, and child care programs.

When to Consider: When a child's social-emotional or behavioral needs are significantly affecting their participation or the group.

What You May Need: Contact your local CCR&R or county health department to find ECMHC services in your area.

Your local contact:

LOCAL COUNTY HEALTH DEPARTMENT

Purpose: Provides a range of health, developmental, and family support services.

Who It Serves: Children and families in the county.

When to Consider: When families need access to health services, WIC, home visiting programs, or other community supports.

What You May Need: Locate your county health department through the NYS DOH website at www.health.ny.gov.

Your local contact:

FAMILY SUPPORT ORGANIZATIONS

Purpose: Community-based programs that provide advocacy, peer support, and family-centered services.

Who It Serves: Children with disabilities, developmental differences, and their families.

When to Consider: When a family wants to connect with other families with similar experiences or needs culturally responsive support.

What You May Need: Organizations vary by region. Ask your CCR&R or school district for local options.

Your local contact:

INCLUSION AND DISABILITY SUPPORT RESOURCES

Purpose: Resources that help programs implement inclusive practices and support children with disabilities in all settings.

Who It Serves: Child care providers and programs.

When to Consider: When you need guidance on accommodations, modifications, or accessible program design.

What You May Need: Contact your regional CCR&R, OCFS, or NYSED for inclusion support resources.

Your local contact:

HANDOUT 1: Objective Observation vs. Opinion

Reference and practice activity for training participants.

What Is an Objective Observation?

An objective observation describes exactly what a child does or says — using factual, specific, measurable language. It includes who, what, when, where, how often, and how long. It does not include interpretations, assumptions, or judgments about the child's motives or character.

What Is an Opinion or Interpretation?

An opinion or interpretation goes beyond what can be directly observed. It assigns meaning, motive, or a label to the child's behavior. Opinions in documentation can be harmful — they can label children unfairly, undermine family trust, and create legal liability.

Side-by-Side Examples

Commonly Used Word	Instead of:	Write this instead:	Why it matters:
Aggressive	He's so aggressive.	When a peer took his block, Marcus pushed the peer and yelled 'mine!' The push lasted 2 seconds.	Describes the observable action without labeling the child.
Lazy	She's lazy and never tries.	During writing time, Amara put her head down for 8 minutes. She did not pick up her pencil when given the direction.	Describes what happened, not what the provider thinks about the child.
Hyper	He's hyper and can't sit still.	During the 15-minute circle time, Elijah left his spot 5 times. He returned with one-to-one adult support.	Provides specific, observable data.
Defiant	She was being defiant again.	When given the clean-up direction, Sofia said 'no' and continued playing. This occurred 3 times over 5 minutes.	Removes judgment and describes the behavior and its duration.

Spoiled	He's so spoiled.	When told it was another child's turn, Liam cried for 6 minutes and said 'I want it!' four times.	Avoids family blame and focuses on what occurred.
Does not listen	She never listens.	When given a 3-step direction, Ana completed step 1 and then began playing. She did not complete steps 2 or 3.	Specific and observable — useful for planning support.
Sensory issues	He has sensory issues.	During art, Carlos pulled his hands away from the paint and said 'yucky' four times. He asked to wash his hands.	Describes what was observed without diagnosing.
Delayed	She seems delayed.	During a 30-minute free play period, Nadia communicated using gestures and 3–4 single words. She did not use two-word phrases during this observation.	Provides data that can be documented and shared with specialists.

Practice Activity

Rewrite each opinion statement as an objective observation. Use specific, factual language.

1. *"She always has a meltdown."*

Rewrite:

2. *"He acts like a baby."*

Rewrite:

3. *"She's manipulative."*

Rewrite:

4. *"He's a problem child."*

Rewrite:

HANDOUT 2: Child Observation Documentation Template

Print and use this template to document observations of individual children.

⚠ **Document facts, not assumptions. Describe what you see and hear. Do not diagnose.**

Child's Name or Initials:

Age:

Classroom or Group:

Date:

Time:

Observer:

Activity or Routine Being Observed

Activity:

ABC Observation Record

A — Antecedent: What happened immediately before?:

B — Behavior: What did the child do or say? (Use exact words and actions):

C — Consequence or Response: What happened after? How did adults respond?:

Additional Details

Duration or frequency of behavior:

Possible pattern noticed (if any):

Strategies tried during or after this incident:

Child's response to support or strategy:

Follow-up needed:

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HANDOUT 3: Conversation Planning Guide

A step-by-step guide for preparing to speak with families.

BEFORE THE CONVERSATION

Gather your documentation: Review your observation notes. Select 2–4 specific, dated examples.

List the child's strengths: Identify at least 3 genuine strengths you will share first.

Clarify your purpose: Are you sharing information? Seeking family input? Suggesting a referral? Know your goal.

Choose the right setting: Request a private meeting — not drop-off or pick-up. Allow adequate time.

Consider the family: Reflect on language, culture, family circumstances, and any prior history.

Arrange interpretation: If needed, arrange for a qualified interpreter. Do not use children as interpreters.

WHAT TO BRING

Observation documentation: Copies of your dated, objective observation notes.

Strengths list: Your written list of what the child does well.

Resource list: Referrals or supports you may recommend.

Conversation Planning Worksheet: Your completed planning worksheet.

Pen and notepad: For taking notes during the conversation.

HOW TO OPEN THE CONVERSATION

Start with appreciation: "Thank you for taking the time to meet with me today."

State your purpose warmly: "I wanted to share some observations and hear what you are noticing at home."

Lead with strengths: "I always love having [child's name] in our group. Here are some things I really appreciate about [him/her/them]..."

HOW TO SHARE CONCERNS RESPECTFULLY

Use objective language: Share your documented observations — not labels or judgments.

Focus on patterns: "Over the past few weeks, we have noticed..."

Avoid diagnosis language: You are sharing what you see, not what it means medically.

Use 'I' statements: "I have noticed..." rather than "Your child is..."

HOW TO INVITE FAMILY INPUT

Ask open-ended questions: "What do you see at home?" and "What works well for your child?"

Listen without interrupting: Give families time to respond. Silence is okay.

Validate their experience: "That makes sense." "Thank you for sharing that with me."

HOW TO RESPOND TO EMOTIONS

Worry or fear: "It makes sense that this feels worrying. Let's take this one step at a time."

Denial: "I understand this may be surprising. I'd love to keep the conversation open."

Anger: Stay calm. "I hear you. I want to work with you, not against you."

Sadness: Give space. "Please take all the time you need. I'm here with you."

HOW TO DISCUSS NEXT STEPS

Be specific: "Here is what I will do in the program..."

Invite collaboration: "What would you like to try at home?"

Mention referrals gently: "There is a program called Early Intervention that offers free evaluations for children under 3. Would you like more information?"

Make a plan: "Let's plan to check in again on [date]."

HOW TO CLOSE THE CONVERSATION

Summarize agreements: Briefly repeat what was discussed and agreed upon.

Express partnership: "I am really glad we had this conversation. I'm committed to supporting your child."

Confirm follow-up: "I'll send home a note with the information we discussed. Let's connect again on [date]."

Thank the family: "Thank you for your time and for trusting me with your child."

HANDOUT 4: Strengths-Based Language Cheat Sheet

A quick reference for professional, family-centered communication.

STARTING WITH STRENGTHS

- "One thing I really love about [child's name] is..."
- "I've noticed that [he/she/they] really excels at..."
- "Families always know their child best, and I'd love to hear what you've seen."
- "I'm so grateful to have [child's name] in our program."

SHARING OBSERVATIONS

- "Over the past few weeks, I've noticed..."
- "I've been documenting some patterns I'd like to share with you."
- "Here is what I observed, along with the date and situation."
- "I want to be transparent with you about what I'm seeing."

ASKING FAMILY-CENTERED QUESTIONS

- "What does this look like at home?"
- "What works really well for your child?"
- "What does [he/she/they] love most?"
- "Is there anything going on that might be helpful for me to know?"
- "What does your child say or do when [he/she/they is] upset at home?"

EXPRESSING CONCERN WITHOUT BLAME

- "I'm sharing this because I care about your child's wellbeing."
- "My goal is always to support your child — not to worry you."
- "I want to work with you as a team."
- "This is not about anything wrong with your child — every child has areas where they grow."

DISCUSSING SUPPORT STRATEGIES

- "Here are some things we've tried that have helped."
- "I'd love to hear if any of these strategies work at home."
- "Let's think together about what might help."
- "Would it be okay if we tried [strategy] and checked in next week?"

RECOMMENDING NEXT STEPS

- "There is a free program that might be a good fit for your family."
- "I'd like to share some information about [Early Intervention / CPSE]. Would that be okay?"
- "Your pediatrician is a great first step — here is what I'd suggest mentioning."
- "Let's set up a time to talk again and see how things are going."

RESPONDING TO PARENT WORRY, DENIAL, ANGER, OR SADNESS

- "It makes complete sense that you feel that way."
- "I hear you. Your feelings are valid."
- "I understand this may feel surprising. I'm here to work with you."
- "You don't have to make any decisions today. Let's take this one step at a time."
- "I will follow your lead. What feels right to you?"

SAY THIS, NOT THAT

Instead of saying...	Try saying...
<i>"Your child is aggressive."</i>	<i>"We've noticed that when another child takes a toy, he sometimes pushes or yells. We want to help him learn safe ways to express frustration."</i>
<i>"She has sensory issues."</i>	<i>"We've noticed that loud sounds and messy textures seem uncomfortable for her. We'd like to talk about what helps her feel calm and comfortable."</i>
<i>"He's defiant."</i>	<i>"We've noticed that certain transitions are harder for him. We've been documenting what helps and would love to hear what works at home."</i>
<i>"She's delayed."</i>	<i>"We've noticed some areas where she may benefit from additional support. We'd like to share what we've observed and explore some options together."</i>
<i>"He doesn't listen."</i>	<i>"We've noticed that he sometimes needs extra support following multi-step directions. We have some ideas we'd like to try and hear your thoughts."</i>
<i>"She's just spoiled."</i>	<i>"We've noticed that waiting for her turn is currently a growing edge for her. We're working on some strategies to support her with this."</i>

HANDOUT 5: Local Referral and Support Resource List — New York State

Customizable for your program, county, and community.

Programs should customize this list by adding local contact information. This handout may be shared with families in partnership with your program director. Contact information for referrals should always be current and verified.

EARLY INTERVENTION (EI) | Birth - 36 Months

Provides free early intervention services for infants and toddlers with developmental delays or disabilities.

To refer: Contact your local EI program. You may refer a child without parent consent, but must notify the family.

Website: www.health.ny.gov/community/infants_children/early_intervention

Local contact name:

Phone:

Email:

Website / notes:

CPSE | Ages 3-5

The Committee on Preschool Special Education provides evaluation and special education services.

To refer: Contact your local school district's CPSE Chairperson. Parent consent is required for evaluation.

Website: www.nysed.gov/special-education

Local contact name:

Phone:

Email:

Website / notes:

PEDIATRICIAN OR HEALTH CARE PROVIDER | All Ages

First point of contact for developmental screening and medical referrals.

Suggest families discuss your observations at their child's next well-child visit.

Local contact name:

Phone:

Email:

Website / notes:

LOCAL SCHOOL DISTRICT | Ages 3-21

Responsible for evaluation and services under CPSE and CSE (Committee on Special Education).

Families can request an evaluation directly from their home school district.

Local contact name:

Phone:

Email:

Website / notes:

COUNTY HEALTH DEPARTMENT | All Ages

Provides public health services, home visiting programs, and family support.

Website: www.health.ny.gov — search by county

Local contact name:

Phone:

Email:

Website / notes:

CHILD CARE R&R AGENCY | All Ages

Connects providers and families to training, coaching, and local resources.

Website: www.ocfs.ny.gov — find your regional CCR&R

Local contact name:

Phone:

Email:

Website / notes:

OCFS RESOURCES | Providers

Regulations, training, and guidance for NYS child care programs.

Website: www.ocfs.ny.gov

Local contact name:

Phone:

Email:

Website / notes:

MENTAL HEALTH CONSULTATION (ECMHC) | All Ages

Early Childhood Mental Health Consultation supports children and families with social-emotional concerns.

Contact your CCR&R or county health department to locate services in your area.

Local contact name:

Phone:

Email:

Website / notes:

FAMILY SUPPORT ORGANIZATIONS | All Ages

Community-based advocacy, peer support, and culturally responsive family services.

Ask your CCR&R or school district for local organizations.

Local contact name:

Phone:

Email:

Website / notes:

Program-Specific Contacts (Complete before distributing)

Program Director:

CPSE Chairperson (Local School District):

Local Early Intervention Contact:

County Health Department:

Child Care R&R Agency:

Preferred Mental Health Consultant:

Additional Community Resource 1:

Additional Community Resource 2:

CONFIDENTIALITY: This record contains personally identifiable information. Store securely, share only with authorized personnel, and follow your program's confidentiality policies and NYS OCFS regulations.